CLINICAL CASE PRESENTATION- OBSTETRICS & GYNAECOLOGY

MELWIN MABEN
FINAL YEAR MBBS
HIMS, HASSAN

Name: Mrs. XYZ
Age: 32 years
Sex: Female
Address: Vidyanagar, Hassan
Occupation: Housewife
Education: PUC
Husband's Details: Mr. XYZ, 38 years, Company worker, Educated up to Degree.
Socioeconomic status: Class 2 of Modified BG Prasad Classification
DOA: 8th May 2019
DOE: 6th March 2019
LMP: 31st August 2018
EDD: 7th June 2019
Obstetric Score: $G_3P_2L_2A_0$

CHIEF COMPLAINTS

- Admitted with reduced growth of fetus on serial ultrasound since last 4 weeks

HISTORY OF PRESENT ILLNESS

- Patient was told to have reduced growth of fetus on serial ultrasound since last 4 weeks and was admitted for close monitoring.
- She is perceiving fetal movements well
- No h/o leaking per vaginum
- No h/o bleeding PV

HISTORY OF PRESENT PREGNANCY

First Trimester:

-Spontaneous Conception

-Pregnancy was confirmed after 1 ½ months of missed period by UPT and she was a booked case at HIMS.

-Regular ANC visits

-Booster dose of Td vaccine taken

-Folic Acid tablets were taken

-First Trimester (Dating Scan) was done and was told normal

No h/o excessive vomiting.

No h/o fever with rashes, excessive vomiting.

No h/o spotting or bleeding per vagina.

No h/o pain abdomen.

No h/o exposure to radiation and drug intake.

- **Second Trimester:**

-Quickening was felt at 5 months of gestation
-Second Trimester(Anomaly Scan) was done and no anomalies were found.
-Iron and Calcium Tablets were taken
-No History of fever with rashes
-No History of any drug intake or radiation exposure
-No History of Per vaginal bleed
-No H/o Pedal edema or headache
-No H/o Raised BP Recording or Raised Blood Sugar recording

Third Trimester:

- Fetal movements are well perceived
- -Iron and Calcium tablets taken
- USG was done at 32 weeks which showed some difference in the growth of the fetus. A repeat scan after 2 weeks was advised, which showed no significant increase in growth of the fetus. She was told that last USG taken 2 days back showed mild changes in the blood supply to the fetus and was advised hospitalization.
- No h/o high Blood pressure or pedal edema
- No h/o abdominal pain.
- No History of Per vaginal bleed/leak per vagina

PAST OBSTETRIC HISTORY

- Married at 20 years of age and concieved spontaneously 3 months after marriage.
- 1st pregnancy:
- No h/o any antenatal complications
- Induced at 40 weeks. Emergency LSCS done in view of fetal distress and meconium staining of liquor . Baby cried after stimulation and admitted in NICU for 2 days.
- Female baby, 2.5 kg, 11 yrs now
- No adverse events in postpartum period
- Baby did well after discharge from ICU, Developmental milestones normal, Presently alive and healthy.
- Breastfed for 1 and ½ yrs.

- 2nd Pregnancy:
- Concieved spontaneously while in lactational amenorrhoea.
- No h/o medical disorders complicating pregnancy.
- H/o reduced growth for the fetus in USG at around 32 weeks of gestation.Regular follow up scans were done and repeat CS was done at 37 weeks in view of changes in blood flow to fetus and scar tenderness.
- Delivered a female baby weighing 2.8kg , cried soon after birth.
- Breastfeeding initiated 1 hr after birth, and was fed till 2 years of age
- No puerperal complications
- Baby's developmental milestones were normal. Presently 9 years.
- Used barrier method of contraception after delivery.

PAST HISTORY

- No h/o DM, HTN, Asthma, TB or cardiac diseases
- No h/o previous surgeries other than LSCS
- No h/o drug allergies
- No h/o previous blood transfusions

MENSTRUAL HISTORY

Menarche: 14 years

Past cycles:

28 days cycle	3-4 days of flow
2-3 pads/day	No H/o Dysmenorrhea, passage of clots

LMP: 31st August 2018

EDD: 7th June 2019

FAMILY HISTORY

- No history bleeding disorder in the family.
- No history of children with chromosomal anomaly /birth defect.

PERSONAL HISTORY

- Diet: Mixed
- Appetite: Normal
- Sleep: Undisturbed
- Bowel and Bladder: Normal and regular
- No H/o any addictions

DIETARY HISTORY

- Non vegetarian diet
- Breakfast : 3 dosas, coconut chutney, 1 cup tea → 550 cal, 6 grams protein
- Midmorning snack: 1 banana, 1 cup milk → 220 cal , 3 grams protein
- Lunch : 2 cups of rice , 1 katori sambar,1 cup pulses,1 piece of fish→ 450 cal, 30 grams protein
- Evening snacks: 1 handful peanuts, 1 cup tea, 2 biscuits→ 320 cal, 20 grams protein
- Dinner : 3 chapathi dressed with ghee, Egg curry → 500 cal , 14 grams protein
- TOTAL → 2000 cal , protein 73 grams

SUMMARY

A 32 year lady with an Obstetric Score of G3P2L2A0 with 9 months of amenorrhea was admitted with complaints of reduced fetal growth seen on serial ultrasounds since 4 weeks. She can perceive fetal movements well and has no other complaints.

GENERAL PHYSICAL EXAMINATION

- Here is a young lady who is moderately built and nourished. She is conscious, cooperative and well oriented to time, place and person.
- Weight – 59 kg
- Height – 150 cm
- BMI – 26.22 kg/m^2
- Pallor, Icterus, Clubbing, Cyanosis, Lymphadenopathy, Edema: Absent
- Breast, Spine, Thyroid: Normal

VITALS

- Pulse rate – 70 beats/min, regular rhythm, normal volume, normal character
- Blood pressure – 120/80 mmHg. Taken from left hand in sitting position
- Respiratory rate – 14 cycles/min, thoraco-abdominal type
- Temperature – 98.6 F

OBSTETRIC EXAMINATION

- INSPECTION

Shape of the abdomen- Distended and appears longitudinally oval.

Flanks: Full

Umbilicus is central and everted.

Linea nigra and Stria gravidarum: Present.

No scars, sinuses or dilated veins can be seen.

Hernial orifices are intact.

- PALPATION
- No local rise of temperature or tenderness.
- Abdominal Girth: 31 inches
- Fundal height: 32 weeks
- Symphysio-fundal height: 31 cm
- Liquor clinically appears mildly reduced
- EFW clinically appears 2 kg

OBSTETRIC GRIPS :

Fundal grip : Soft broad mass felt on suggestive of fetal breech.

Right Lateral grip : Uniform, continuous curved resistance suggestive of spine.

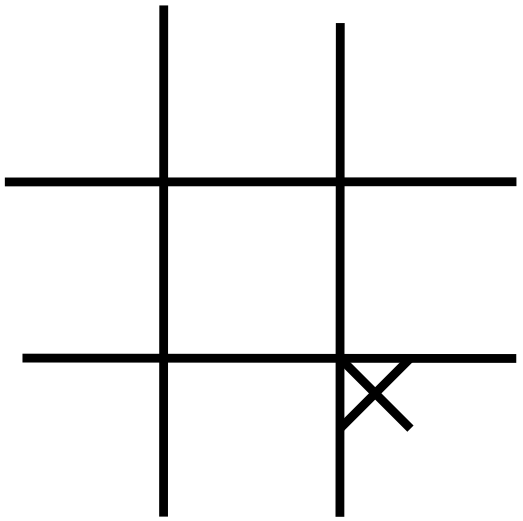
Left lateral grip : Irregular knob like structures suggestive of limb buds.

1st Pelvic grip : Hard, non ballotable globular mass felt suggestive of fetal head

2nd Pelvic grip : Fingers are diverging

AUSCULTATION :

- Fetal Heart sounds heard in the left spino umbilical line – 140 bpm



SYSTEMIC EXAMINATION

Cardiovascular system

S1, S2 sounds heard, no murmurs heard

Respiratory system

Normal vesicular sounds heard

No added sounds

Central nervous system

No focal neurological deficit

SUMMARY

- A 32 year lady with an Obstetric Score of G3P2L2A0 with 9 months of amenorrhea was admitted with complaints of reduced fetal growth seen on serial ultrasounds since 4 weeks. She can perceive fetal movements well and has no other complaints.
- Her vitals are normal and on obstetric examination, a live fetus is palpated with symphysio-fundal height corresponding to 32 weeks of gestation and expected fetal weight of 2 kg with hard, non ballotable mass palpated on First pelvic grip suggestive that the fetus is Cephalic in presentation with non-engaged head. Fetal heart sounds are heard and are normal.

PROVISIONAL DIAGNOSIS

A 32 year old lady with obstetric index $G_3P_2L_2A_0$ with two previous LSCS, with 36 weeks of gestation, with single live fetus, in longitudinal lie, Cephalic presentation, not in labour and is probably a case of Fetal Growth Restriction.